

and other ocular reactions. Hypersensitivity reaction, and blurred vision of liver function tests, impairment of renal function, agranulocytosis and thrombocytopenia have occasionally been observed.

Interactions With Other Medicaments:

Coumarin anticoagulants - Increases risk of bleeding; use with caution.

Aspirin - Decreases anti-inflammatory activity (potential).

Symptoms and Treatment of Overdose:

There is no specific antidote to overdose with ibuprofen. Gastric lavage may be performed and if necessary, blood electrolytes corrected.

Storage Condition:

Store in a dry place (below 30°C). Protect from light.

Pack Size:

Blister pack of 2x10's, 10x10's, 50x10's and 100x10's.

Product Registration Number:

MAL19860999AZ

Further information can be obtained from your doctor or pharmacist.

ROYCE®

Product Holder/Manufactured by:

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ROYCE®

Ibuprofen Film-Coated Tablet 200mg

(Ibuprofen-Anti-Inflammatory analgesic)

Presentation:

A round, shallow biconvex, peach coloured, smooth and glossy tablet which is plain on both sides.

Each tablet contains:

Ibuprofen 200 mg

Pharmacological Information:

Inhibit the biosynthesis of prostaglandins; this action may be the basis of its anti-inflammatory and antipyretic properties.

Ibuprofen, a phenylpropionic acid derivative has analgesic, anti-inflammatory and antipyretic actions. Ibuprofen is absorbed from the gastro-intestinal tract and peak plasma concentrations occur about 1 to 2 hours after ingestion. Ibuprofen is extensively bound to plasma proteins and has ½ life of about 2 hours. It is rapidly excreted in the urine mainly as metabolites and their conjugates. About 1% is excreted in urine as unchanged ibuprofen and about 14% as conjugated ibuprofen.

Indication:

Ibuprofen is indicated for the relief of pain such as headache, dental pain, period pain, rheumatic pain, muscular pain and backache. Ibuprofen relieves fever such as fever associated with cold and flu. Ibuprofen is also indicated for the relief of inflammation of muscles and joints in rheumatic disease including juvenile arthritis and other muscular skeletal disorder.

Dosage and Administration:

Adults and children over 12 years: Initial dose 2 tablets, then, if necessary, 1 or 2 tablets every 4 hours. It is recommended not to exceed 1200 mg daily for minor ailments (eg. fever and headache) but can be used up to a maximum of 2400 mg daily, if prescribed by the doctor. Tablets should be taken with or after food.

Contraindication:

Ibuprofen should not be given to patients who are sensitive to aspirin, ibuprofen or other nonsteroidal anti-inflammatory drugs (NSAIDs). It should not be given to patients in whom aspirin and other NSAIDs induce the symptom of asthma, rhinitis and urticaria. It should not be given to patients with active or history of peptic ulceration.

Warning and Precautions:**Warning:****RISK OF GI ULCERATION,
BLEEDING AND PERFORATION WITH NSAID**

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor GI problem (eg. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms. Studies to date have not identified any subset to patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous report for fatal GI events.

Precautions:

Patients with impaired liver function or a history of liver disease who are on long-term ibuprofen therapy should have hepatic function monitored at regular intervals. Patients on long-term therapy should also have regular haematological monitoring. Patients with a history of cardiac failure should use this drug with caution since fluid retention and oedema have been reported in association with ibuprofen. Ibuprofen can inhibit platelet aggregation. This effect may be enhanced in patients with underlying haemostatic defects; it should be used with care in such patients. If corticosteroids are reduced or discontinued during ibuprofen therapy, the dose should be reduced gradually. As with other drugs of this class, ibuprofen may mask the usual signs of infection.

Cardiovascular Thrombotic Events:

Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk on adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration. There is no consistent evidence that concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.

Hypertension:

NSAID may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAID may have an impaired antihypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation on NSAID treatment and at regular intervals thereafter.

Heart Failure:

Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention heart failure.

Gastrointestinal Events:

All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation, which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events eg the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn the patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions:

NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

Pregnancy and Lactation:**Pregnancy**

Ibuprofen is not recommended for use during pregnancy unless in the judgement of the physician the potential benefits to the mother outweigh the possible hazards to the fetus.

Lactation

Ibuprofen is not recommended for nursing mothers unless the expected benefits to the mother outweigh the potential risk to the neonate.

Side Effects:

The most frequent adverse effects occurring with ibuprofen are gastro-intestinal disturbances. Peptic ulceration and gastrointestinal bleeding have been reported. Other side-effects include headache, dizziness, nervousness, skin rash, pruritis, oedema, depression, drowsiness, insomnia, and blurred vision.